Geographic Variation in the Black-White Life Expectancy Gap in Wisconsin, 2005-2017

Jason Fletcher

University of Wisconsin-Madison

Working Paper -- March 2026

Abstract

Racial disparities in life expectancy persist across the United States, but state-level averages can mask important geographic heterogeneity. This paper examines the Black-White life expectancy gap in Wisconsin from 2005 to 2017, disaggregating the state into Milwaukee County, Dane County (Madison), and the rest of the state. Because Milwaukee is home to roughly 60 percent of Wisconsin's Black population and exhibits the worst Black mortality outcomes, statewide statistics are heavily influenced by conditions in a single county. I use counterfactual mortality-replacement exercises to quantify this composition effect: if Black Milwaukeeans had the same age-specific mortality as Black people elsewhere in the state, the statewide male gap would fall from 7.9 to 4.2 years -- a nearly 50 percent reduction. Meanwhile, Dane County's large gap (8.5 years for males) is partly an artifact of exceptionally high White life expectancy; replacing Dane White mortality with statewide rates reduces the gap by 2.4 years. These findings show that a single statewide disparity metric conflates qualitatively different local dynamics and that policy responses must be geographically targeted.

Keywords: life expectancy, racial disparities, Wisconsin, geographic variation, counterfactual analysis

1. Introduction

The Black-White gap in life expectancy is one of the most persistent health disparities in the United States. Although this gap has narrowed nationally over recent decades, substantial variation remains across states and localities (Harper et al. 2014; Riddell et al. 2018). State-level analyses, while informative, can obscure important within-state heterogeneity driven by residential segregation, differential access to health care, economic inequality, and other social determinants of health.

Wisconsin is a particularly instructive case. The state consistently ranks among the worst for Black-White disparities across a range of social and economic indicators. The Milwaukee metropolitan area has been repeatedly identified as one of the most racially segregated in the nation. Roberts et al. (2019) documented a persistent statewide Black-White life expectancy gap in Wisconsin that showed little improvement between 1999 and 2016, in contrast to national trends of convergence. Critically, however, their analysis was conducted at the state level.

A state-level analysis of Wisconsin is misleading for a specific, quantifiable reason: Milwaukee County alone contains roughly two-thirds of the state's Black population. Wisconsin is not unique in this respect: in many states, a single county holds the largest share of the state's Black population (see Appendix C). If mortality conditions for Black residents of Milwaukee are substantially worse than elsewhere in the state -- as one might expect given the city's concentrated poverty and extreme segregation -- then the statewide Black life expectancy is overwhelmingly a Milwaukee statistic. The "Wisconsin gap" is, to a considerable extent, the Milwaukee gap. Conversely, Dane County (Madison) has experienced rapid growth in its Black population and is often seen as a high-opportunity community, yet casual observation of aggregate statistics may overstate racial equity there.

This paper makes three contributions. First, I disaggregate Wisconsin's Black-White life expectancy gap into three regions -- Milwaukee County, Dane County, and the rest of the state -- revealing striking geographic variation. Second, I conduct counterfactual mortality-replacement exercises that quantify how much of the statewide gap is attributable to Milwaukee's concentrated disadvantage and how much of Dane County's gap reflects its exceptionally high White life expectancy. Third, I document a troubling deterioration in Black male life expectancy in both Dane County and the rest of the state during the latter years of the study period. The analytic approach is validated against the published results of Roberts et al. (2019); this replication is reported in the appendix.

2. Background

A growing literature documents within-state variation in racial life expectancy gaps. Kaufman et al. (2019) showed that Black-White differences in life expectancy varied substantially across four US states from 1969 to 2013. Roesch et al. (2023) found that the Black-White gap differed markedly across three large US cities, ranging from 3.4 to 8.8 years for males. These studies demonstrate that state or national averages are inadequate for understanding local health disparities.

A less-explored dimension of this heterogeneity is the role of population composition. When a state's minority population is geographically concentrated in areas with poor health outcomes, the statewide disparity metric conflates two distinct phenomena: the underlying severity of local health disadvantage and the share of the population exposed to it. A counterfactual framework helps disentangle these. By

hypothetically replacing one region's age-specific mortality rates with those of another while holding population shares constant, one can estimate how much of the statewide gap is attributable to conditions in a specific locality.

This compositional lens is especially relevant for Wisconsin. With approximately two-thirds of its Black population in Milwaukee County (a pattern shared by many states; Appendix C), any statewide Black health statistic is disproportionately a Milwaukee statistic. Understanding this is essential for correctly targeting interventions: statewide programs may dilute resources across regions where the gap is modest, while the community most in need -- Milwaukee -- requires concentrated investment.

Similarly, the interpretation of large Black-White gaps in high-amenity communities like Dane County (Madison) requires care. A large gap can arise from severely disadvantaged Black mortality, from exceptionally advantaged White mortality, or from both. These scenarios have different policy implications: the first calls for investment in Black health infrastructure; the second may simply reflect a comparison group with unusually favorable conditions.

3. Data and Methods

3.1 Data Sources

Mortality data come from individual death records for all deaths in Wisconsin from 2005 to 2017, including year of death, county of residence (FIPS code), age, sex, race, and Hispanic origin. I restrict the analysis to non-Hispanic Black and non-Hispanic White individuals (NCHS Hispanic origin codes < 200). The data contain 614,954 deaths meeting these criteria.

Population denominators come from the CDC's bridged-race population estimates for Wisconsin counties, 2005--2017, by single year of age (0--89, 90+), sex, and race. Hispanic origin is not separately identified in the population data (origin = 9 for all records), so the denominators include Hispanic individuals within each racial category. This is a minor concern for Black populations in Wisconsin, where the Hispanic share is small.

Geographic regions are defined using county FIPS codes: Milwaukee County (55079), Dane County (55025), and the rest of Wisconsin (all other counties).

3.2 Life Table Construction

I construct abridged period life tables following Chiang (1968) and Preston et al. (2001). Deaths and populations are aggregated into 19 age groups: <1, 1--4, 5--9, ..., 80--84, 85+. To stabilize rates for smaller populations, I pool data over three-year windows centered on each year from 2006 to 2016 (e.g., the "2010" estimate uses data from 2009--2011). Both deaths and population are summed across the three-year window, so the resulting rates represent average annual age-specific mortality. Infant nax values use Coale-Demeny formulas from Preston et al. (2001, Table 3.3); the open-ended interval (85+) uses nax = 1/M.

3.3 Counterfactual Framework

To decompose the sources of geographic variation, I conduct three counterfactual exercises. In each, I replace one group's set of 19 age-specific mortality rates with those of a donor region, hold the target population's age composition and size constant, and reconstruct the relevant life table. These are purely mechanical exercises -- they quantify the arithmetic importance of particular mortality schedules, not causal effects.

Exercise 1: I replace Milwaukee's Black age-specific mortality rates with those of the rest of Wisconsin's Black population, then recompute the statewide Black life table. This answers: how much of the statewide gap is driven by Milwaukee's elevated Black mortality? Exercise 2 substitutes Dane County's Black mortality rates for Milwaukee's, as an alternative benchmark. Exercise 3 asks a different question about Dane County: I replace Dane County's White mortality rates with those of the rest of the state, to estimate how much of Dane's gap reflects its exceptionally healthy White population rather than its Black mortality disadvantage.

3.4 Formal Gap Decomposition

To provide a comprehensive accounting of the statewide gap, I also decompose it into geography-specific contributions. If life expectancy is approximately a population-weighted average of

sub-regional values, then the statewide Black-White gap can be written as:

$$Gap_SW = Sum_g\,[\,pi_g \times gap_g\,] + White_composition + residual$$

where pi_g is the share of the statewide Black population living in geography g, gap_g is the local Black-White life expectancy gap in geography g, the White composition term captures the fact that the statewide White life expectancy is weighted by White population shares (which differ from Black shares), and the residual reflects the nonlinearity of life expectancy aggregation. Each geography's contribution (pi_g x gap_g) is the product of a composition weight (how many Black people live there) and a place-specific gap (how large the local disparity is). This decomposition quantifies how much of the statewide gap is attributable to each region and enables a clean separation of "place" and "composition" effects.

To isolate the composition effect, I compare the actual weighted sum of local gaps (using Black population shares) with the sum that would obtain if Black people had the same geographic distribution as White people. The difference measures how much the concentration of Black residents in high-gap regions inflates the statewide disparity beyond what geography-specific health conditions alone would produce.

3.5 Bootstrap Confidence Intervals

To quantify uncertainty in life expectancy and gap estimates, I use a parametric bootstrap. For each of 500 replicates, I resample death counts in each age-geography-race-sex cell from a Poisson distribution with mean equal to the observed death count, hold population denominators fixed, and recompute all life tables. Reported 95% confidence intervals are the 2.5th and 97.5th percentiles of the bootstrap distribution. This approach reflects sampling uncertainty in death counts; estimates for small populations (e.g., Dane County Black residents) are less precise.

4. Results

4.1 Geographic Variation in the Black-White Gap

Table 1 presents life expectancy and the Black-White gap for the most recent period (2015--2017) across the four geographic units. The results reveal substantial heterogeneity masked by the statewide average. (Statewide trends are validated against Roberts et al. 2019 in Appendix B.)

Table 1. Life expectancy at birth (years) by race, sex, and geography, 2015--2017 (3-year pooled).

Geography	Sex	Black LE	White LE	B-W Gap
Dane	Female	78.30	83.73	5.43
Dane	Male	71.71	80.21	8.50
Milwaukee	Female	75.95	82.05	6.10
Milwaukee	Male	68.25	77.48	9.23
Rest of WI	Female	77.81	82.13	4.32
Rest of WI	Male	73.94	77.78	3.84
Statewide	Female	76.53	82.25	5.72
Statewide	Male	70.03	77.95	7.92

For males, the Black-White gap ranges from 3.8 (95% CI 2.6--4.9) years in the rest of Wisconsin to 9.2 (95% CI 8.6--9.9) years in Milwaukee County -- a nearly 2.5-fold difference. Dane County's male gap (8.5 (95% CI 1.4--10.6) years) is nearly as large as Milwaukee's, despite Dane County's substantially higher overall life expectancy. This reflects a dual pattern: White males in Dane County enjoy the highest life expectancy of any group (80.2 years), but Black males in Dane County do not share proportionally in this advantage (71.7 years). Milwaukee has the lowest Black male life expectancy of any region (68.3 years), which is 5.7 years below the rest-of-state average for Black males.

For females, the geographic pattern is less dramatic but still notable. The gap ranges from 4.3 (95% CI 3.0--5.5) years (rest of Wisconsin) to 6.1 (95% CI 5.4--6.7) years (Milwaukee). Dane County shows the smallest female gap (5.4 (95% CI 3.3--7.3) years), driven by relatively high Black female life expectancy there (78.3 years, the highest of any region). A caveat is warranted: Dane County's Black population is small relative to the other regions (roughly 50,000 over the 3-year window, with approximately 260 male and 158 female Black deaths), which limits the precision of these estimates.

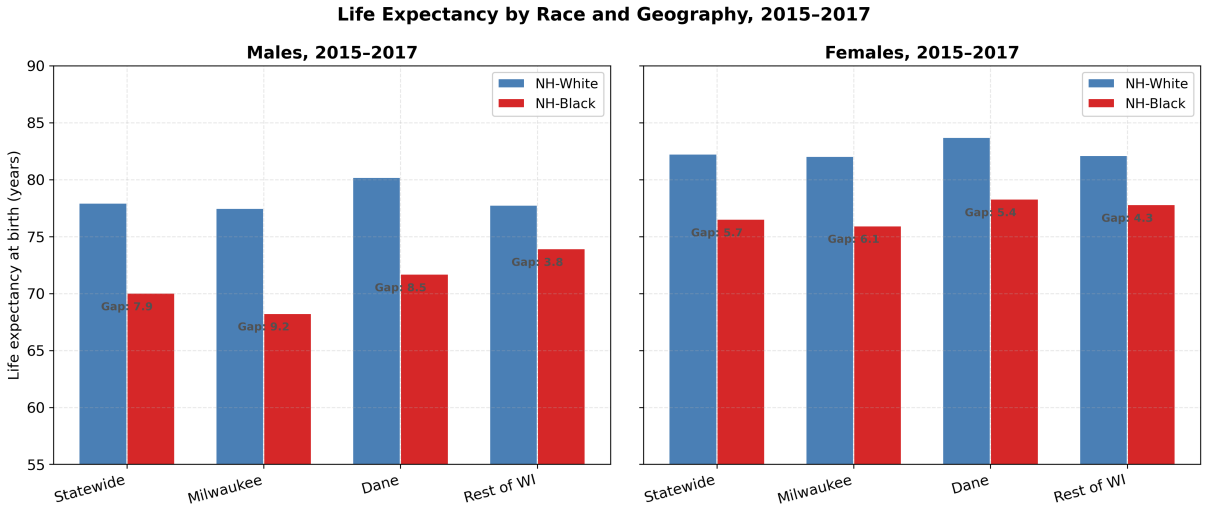

Figure 1. Life expectancy at birth by race and geography, males (left) and females (right), 2015--2017.

4.2 Trends in the Gap by Geography

Figures 2 and 3 plot the Black-White gap over time for each geography. The most striking finding is the divergent trajectory of the male gap across regions. In Milwaukee, the male gap narrowed from 8.6 years (2006) to a low of 7.1 years (2010) before widening sharply to 9.2 years by 2016. In Dane County, the male gap nearly doubled from 5.3 years (2006) to 8.5 years (2016). In the rest of Wisconsin, the gap remained comparatively small but also widened, from 2.1 to 3.8 years.

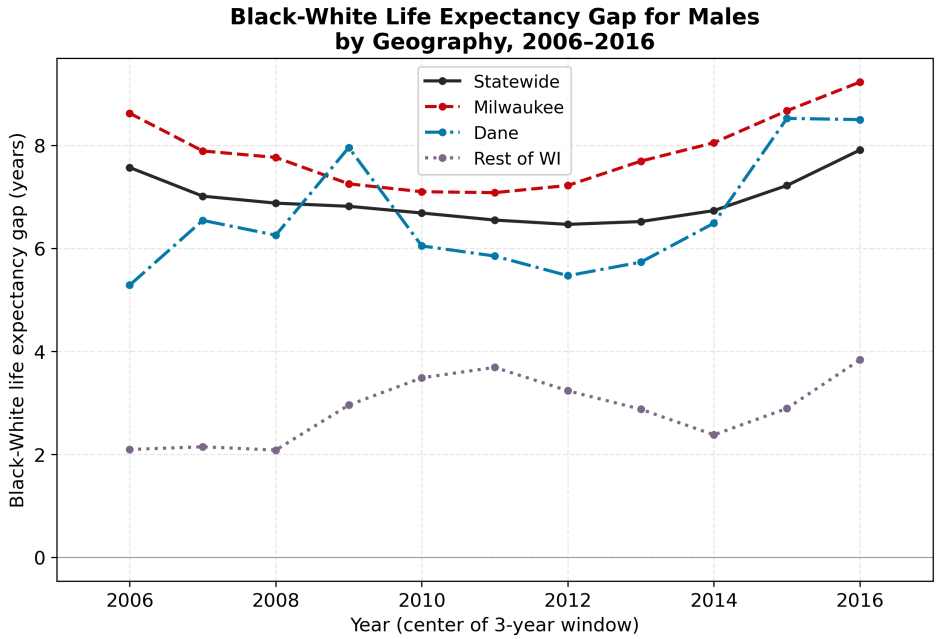

Figure 2. Black-White life expectancy gap for males by geography, 2006--2016 (3-year pooled estimates).

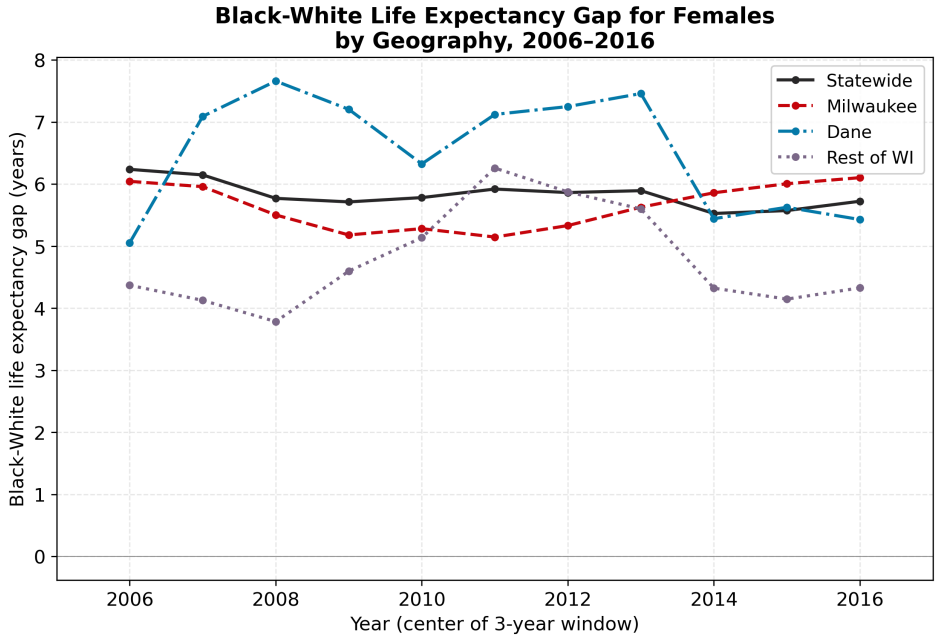

Figure 3. Black-White life expectancy gap for females by geography, 2006--2016 (3-year pooled estimates).

For females, trends were more stable. The statewide female gap narrowed slightly from 6.2 to 5.7 years. Milwaukee's female gap remained roughly constant near 6 years. Dane County showed considerable volatility -- likely reflecting the small Black female population -- but ended the period at 5.4 years, similar to where it began.

4.3 Decomposing the Change Over Time

Table 2 decomposes the change in the gap between the earliest (2005--2007) and latest (2015--2017) periods into changes in White and Black life expectancy. A widening gap could reflect White gains, Black losses, or both -- and the pattern differs sharply by geography.

Table 2. Change in the Black-White life expectancy gap (years) between 2005--2007 and 2015--2017, with decomposition into White and Black LE changes.

Geography	Sex	Gap (05--07)	Gap (15--17)	Change	Delta White LE	Delta Black LE
Statewide	Male	7.57	7.91	0.34	0.55	0.20
Statewide	Female	6.24	5.72	-0.52	0.20	0.72
Milwaukee	Male	8.62	9.23	0.61	1.13	0.52
Milwaukee	Female	6.04	6.10	0.06	0.77	0.71
Dane	Male	5.29	8.50	3.21	0.85	-2.37
Dane	Female	5.05	5.43	0.38	0.46	0.08
Rest of WI	Male	2.10	3.84	1.74	0.41	-1.33
Rest of WI	Female	4.37	4.33	-0.04	0.07	0.12

In Dane County, the male gap widened by 3.2 years -- the largest increase of any region-sex combination -- driven primarily by a 2.4-year decline in Black male life expectancy (from 74.1 to 71.7 years), while White male life expectancy increased by 0.9 years. In Milwaukee, the male gap widened by 0.6 years, with both groups gaining but White males gaining faster (+1.1 vs. +0.5 years). In the rest of Wisconsin, Black male life expectancy declined by 1.3 years, widening the gap by 1.7 years. Among females, the statewide gap narrowed by 0.5 years, driven by faster improvement among Black females (+0.7 years) relative to White females (+0.2 years).

4.4 Decomposing the Statewide Gap

The formal decomposition reveals that the statewide gap is overwhelmingly driven by Milwaukee. Table 3 presents the decomposition for three key periods and both sexes. Each geography's contribution equals the product of its Black population share (pi) and its local B-W gap. The contributions sum, with a small White composition adjustment, to the total statewide gap.

Table 3. Decomposition of the statewide B-W gap (years). Each geography's contribution = pi_g (Black pop. share) x local B-W gap. Percentages show share of total gap. Adj. = White composition + aggregation residual.

Year	Sex	Gap	Milwaukee	Dane	Rest of WI	Adj.
2006	Male	7.57	5.67 (75%)	0.38 (5%)	0.57 (7%)	0.95
2006	Female	6.24	4.45 (71%)	0.33 (5%)	0.87 (14%)	0.59
2010	Male	6.69	4.53 (68%)	0.47 (7%)	0.99 (15%)	0.69
2010	Female	5.78	3.78 (65%)	0.46 (8%)	1.08 (19%)	0.46
2016	Male	7.91	5.58 (71%)	0.72 (9%)	1.19 (15%)	0.42
2016	Female	5.72	4.15 (72%)	0.43 (7%)	1.05 (18%)	0.10

For males in 2016, Milwaukee contributes 5.6 years of the 7.9 (95% CI 7.4--8.4)-year statewide gap -- 70 percent of the total. This dominance reflects both Milwaukee's large Black population share (about two-thirds) and its large local gap (9.2 (95% CI 8.6--9.9) years). Dane County's gap is nearly as large (8.5 (95% CI 1.4--10.6) years), but because only 8.5 percent of the state's Black males live there, its contribution is just 0.7 years (9 percent). The rest of Wisconsin contributes 1.2 years (15 percent), with a moderate gap (3.8 years) offset by a moderate population share (31 percent). The White composition adjustment is small (0.4 years combined), confirming that the decomposition is driven almost entirely by the interplay of Black population shares and local gaps.

For females, Milwaukee's contribution is also dominant (4.1 years, 73 percent of the 5.7-year gap in 2016), though the rest of Wisconsin contributes a larger share (18 percent) than for males, reflecting the fact that the female gap is more geographically uniform.

Figure 4 visualizes this decomposition over time. Each stacked bar shows how the statewide gap is built from geography-specific contributions. Milwaukee's red segment dominates in every period, but its share of the total has fluctuated. For males, the widening gap after 2012 reflects growing contributions from all three regions, with both Milwaukee's and the rest of Wisconsin's segments expanding.

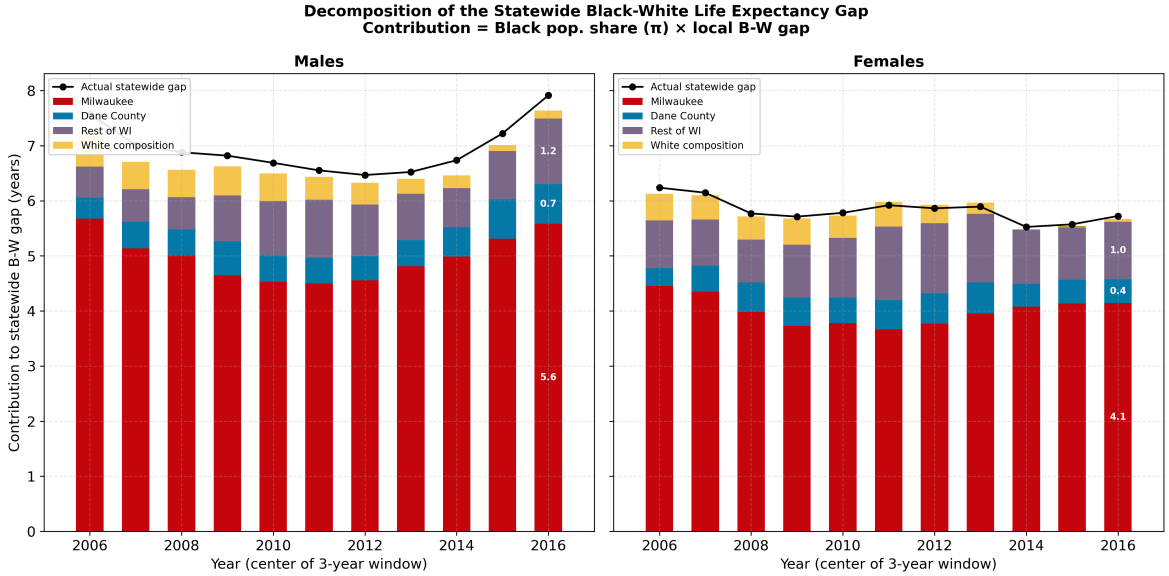

Figure 4. Decomposition of the statewide Black-White life expectancy gap into geographic contributions (pi x local gap) for males (left) and females (right), 2006--2016.

The composition effect -- how much the geographic concentration of Black residents in high-gap areas inflates the statewide gap -- is substantial. If Black Wisconsinites had the same geographic distribution as White Wisconsinites, the weighted sum of local male gaps would be 4.9 years instead of 7.5 years in 2016: the composition penalty is 2.6 years. In other words, roughly a third of what drives the statewide male gap is not the severity of local disparities per se, but the fact that most Black residents live in the worst-performing region.

Figure 5 illustrates the two components of each geography's contribution: the Black population share (Panel A) and the local B-W gap (Panel B). Milwaukee's contribution is large because it ranks highest on both dimensions. Dane County's high local gap is offset by its small Black population share, keeping its statewide contribution modest.

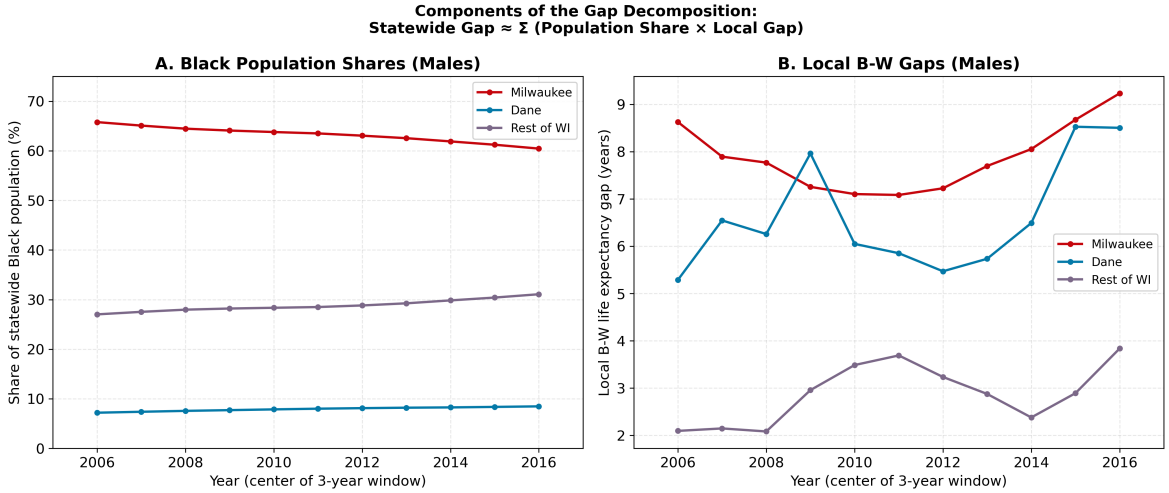

Figure 5. Components of the gap decomposition for males: (A) Black population shares by geography and (B) local B-W life expectancy gaps by geography, 2006--2016.

4.5 Counterfactual Exercises

The decomposition quantifies each region's contribution to the statewide gap; the counterfactual exercises below ask what the gap would be under specific alternative mortality schedules.

Table 4 presents Exercise 1: what if Black Milwaukeeans had the same age-specific mortality rates as Black people in the rest of Wisconsin? In 2016, the statewide male gap would fall from 7.9 (95% CI 7.4--8.4) to 4.2 years -- a reduction of 3.8 years, or nearly half the observed gap. Black male life expectancy would rise from 70.0 to 73.8 years. For females, the gap would narrow from 5.7 to 4.4 years. This is consistent with the decomposition finding that Milwaukee contributes 70 percent of the statewide male gap.

Table 4. Counterfactual Exercise 1: statewide B-W gap if Milwaukee's Black population had Rest-of-WI Black age-specific mortality.

Year	Sex	Actual Gap	CF Gap	Reduction	Actual Black e0	CF Black e0
2006	Male	7.57	2.19	5.38	69.83	75.21
2006	Female	6.24	4.32	1.91	75.81	77.73
2010	Male	6.69	3.64	3.05	71.38	74.43
2010	Female	5.78	5.16	0.62	76.87	77.49
2016	Male	7.91	4.17	3.75	70.03	73.78
2016	Female	5.72	4.37	1.35	76.53	77.88

Exercise 2 asks: what if Milwaukee's Black population instead had Dane County's Black mortality? In 2006, this would have reduced the male gap by 4.6 years (from 7.6 to 3.0), since Dane County Black males had relatively favorable life expectancy at that time (74.1 years). But by 2016, this exercise yields a smaller reduction of only 2.3 years (from 7.9 to 5.6), because Dane County Black male mortality had itself deteriorated. This time comparison underscores that the Black male mortality crisis extends beyond Milwaukee.

Exercise 3 addresses a different question: why is Dane County's gap so large despite its reputation as a progressive, high-opportunity community? Part of the answer is that the comparison group -- Dane County Whites -- is exceptionally healthy. Table 5 shows what happens when Dane County White mortality is replaced with Rest-of-WI White mortality.

Table 5. Counterfactual Exercise 3: Dane County B-W gap if Dane Whites had Rest-of-WI White age-specific mortality.

Year	Sex	Actual Gap	CF Gap	Reduction	Actual White e0	CF White e0
2006	Male	5.29	3.29	2.00	79.36	77.37
2006	Female	5.05	3.84	1.21	83.27	82.06
2010	Male	6.05	4.32	1.73	79.76	78.04
2010	Female	6.32	5.03	1.30	83.91	82.61
2016	Male	8.50	6.07	2.43	80.21	77.78
2016	Female	5.43	3.83	1.59	83.73	82.13

In 2016, replacing Dane White mortality with Rest-of-WI rates reduces the male gap from 8.5 to 6.1 years and the female gap from 5.4 to 3.8 years. Roughly one-quarter to one-third of Dane County's gap

is attributable to the exceptionally favorable White mortality environment. The remaining gap of 6.1 years for males is still substantial and reflects genuinely disadvantaged Black mortality in Dane County, particularly the 2.4-year decline in Black male life expectancy documented in Table 2. Thus Dane County's large gap is driven by both an unusually healthy White benchmark and a deteriorating Black health trajectory.

5. Discussion

This analysis yields four principal findings. First, the statewide Black-White life expectancy gap in Wisconsin masks a nearly 2.5-fold range across regions: from 3.8 (95% CI 2.6--4.9) years in the rest of the state to 9.2 (95% CI 8.6--9.9) years in Milwaukee for males. Second, the formal decomposition reveals that Milwaukee contributes 70 percent of the statewide male gap -- a dominance driven by the combination of its large Black population share (about two-thirds) and its large local gap (9.2 (95% CI 8.6--9.9) years). The geographic concentration of Black Wisconsinites in Milwaukee adds approximately 2.6 years to the statewide male gap compared to a scenario in which Black and White residents had the same geographic distribution. Third, the gap widened in all regions for males between 2005-2007 and 2015-2017, with the most dramatic deterioration in Dane County, where Black male life expectancy actually declined by 2.4 years. Fourth, Dane County's large gap is partly a mechanical consequence of its exceptionally healthy White population -- roughly a quarter to a third of the gap reflects this benchmark effect rather than Black disadvantage per se.

These findings have important implications for how we interpret and respond to state-level racial health disparities. The "Wisconsin gap" is not a diffuse, statewide phenomenon amenable to generic state-level programs. The decomposition shows that 70 percent of the statewide male gap (7.9 (95% CI 7.4--8.4) years) traces to a single county where about two-thirds of the state's Black population lives under mortality conditions that produce a male life expectancy of 68.3 years -- below the US national average for Black males and 5.7 years below the Black male average in the rest of the state. Statewide interventions that spread resources uniformly are poorly matched to this geographic concentration of disadvantage.

The Dane County finding deserves particular attention. Dane County is widely perceived as a progressive, high-opportunity community with strong public institutions and a robust economy. Yet the Black-White male life expectancy gap there (8.5 (95% CI 1.4--10.6) years) is nearly as large as Milwaukee's, and it widened faster. The counterfactual decomposition reveals that this gap has two sources: the exceptional longevity of Dane County's White population (which inflates the reference standard by 2.4 years) and a genuine decline in Black male life expectancy. The latter is the more actionable concern. Favorable average conditions do not automatically produce equitable health outcomes. Social determinants driving mortality disparities may operate through mechanisms -- residential segregation within the county, differential incarceration, concentrated poverty -- that are not captured by county-level aggregates.

The decline in Black male life expectancy in several regions during the latter years of the study period is consistent with emerging national trends linked to the opioid epidemic, increases in homicide, and rising "deaths of despair." Cause-of-death decomposition (e.g., Arriaga 1984) is needed to identify which specific causes are driving these reversals and to inform targeted interventions.

Several limitations should be noted. The population denominators do not separately identify Hispanic origin, which may introduce a small upward bias in population counts for non-Hispanic groups. Reported 95% confidence intervals from a parametric bootstrap (Section 3.5) reflect sampling uncertainty in death counts; Dane County's Black population, while growing, remains relatively small, and the 3-year pooling mitigates but does not eliminate instability in those estimates. The counterfactual exercises are mechanical decompositions, not causal estimates -- they quantify the arithmetic contribution of regional

mortality schedules but do not identify why those schedules differ. The analysis covers 2005--2017 and does not capture more recent trends, including the COVID-19 pandemic, which disproportionately affected Black life expectancy nationwide. Finally, this analysis does not decompose the gap by cause of death, which would help identify targeted intervention points.

6. Conclusion

Statewide life expectancy statistics, while valuable, can obscure the geographic concentration of racial health disparities. In Wisconsin, the Black-White life expectancy gap (7.9 (95% CI 7.4--8.4) years for males) varies by a factor of nearly 2.5 across regions, and the trajectories are diverging. A formal decomposition reveals that Milwaukee alone accounts for 70 percent of the statewide male gap, driven by the double burden of a large Black population share and severe local mortality disadvantage. Black geographic concentration adds roughly 2.6 years to the statewide male gap -- a quantifiable composition penalty. Dane County's gap (8.5 (95% CI 1.4--10.6) years), meanwhile, is inflated by exceptionally high White life expectancy but also reflects a genuine and alarming decline in Black male life expectancy. Policy responses must be geographically targeted: Milwaukee's crisis requires investment in the structural determinants of Black health in a hyper-segregated urban environment, while Dane County's growing disparity calls for investigation of why a seemingly prosperous community is failing its Black residents.

Appendix A: Geographic Reference

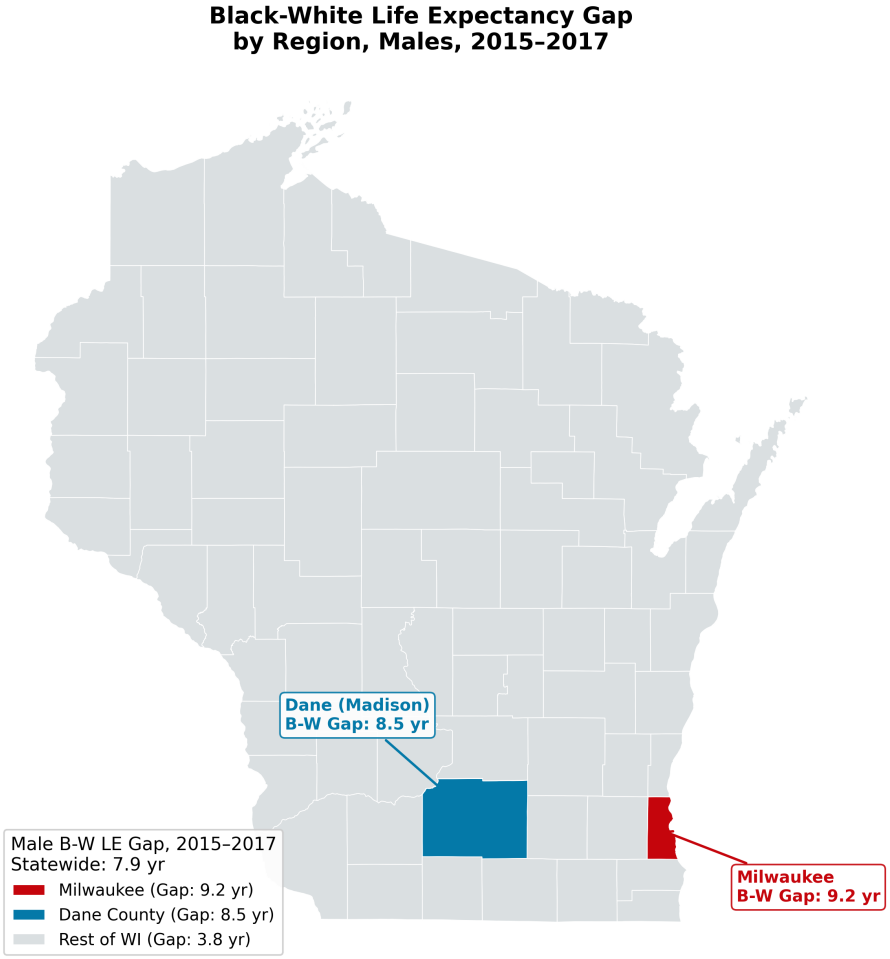

Figure A1. Wisconsin counties with Milwaukee, Dane, and rest-of-state regions highlighted. Annotations show male Black-White life expectancy gap for 2015--2017.

Appendix B: Replication of Roberts et al. (2019)

To validate the analytic pipeline, I replicate the statewide life expectancy estimates of Roberts et al. (2019) for the 2014--2016 period (center year 2015). Table B1 compares my estimates with the published values. My estimates are within 0.1 to 0.4 years of the published values across all race-sex groups. The small discrepancies likely reflect differences in Hispanic-origin filtering and the exact population denominators used. The Black-White gap in my estimates is 7.2 years for males and 5.6 years for females, compared to 7.3 and 5.6 years in Roberts et al.

Table B1. Replication check: statewide life expectancy (years) for 2014--2016, compared with Roberts et al. (2019).

Group	Roberts et al.	This Paper	Difference
White Males	77.75	78.06	+0.31
Black Males	70.41	70.84	+0.43
White Females	82.15	82.24	+0.09
Black Females	76.54	76.67	+0.13
Male B-W Gap	7.34	7.22	-0.12
Female B-W Gap	5.61	5.57	-0.04

Figures B1 and B2 show statewide life expectancy trends for males and females, respectively. Among males, White life expectancy was essentially flat (77.4 to 78.0 years), while Black male life expectancy rose from 69.8 to a peak of 71.6 years around 2012 before declining to 70.0 years by 2016. This reversal widened the male gap from 6.5 years at its narrowest (2012) to 7.9 years at the end of the study period. Female trends were more stable, with the gap fluctuating between 5.6 and 6.2 years.

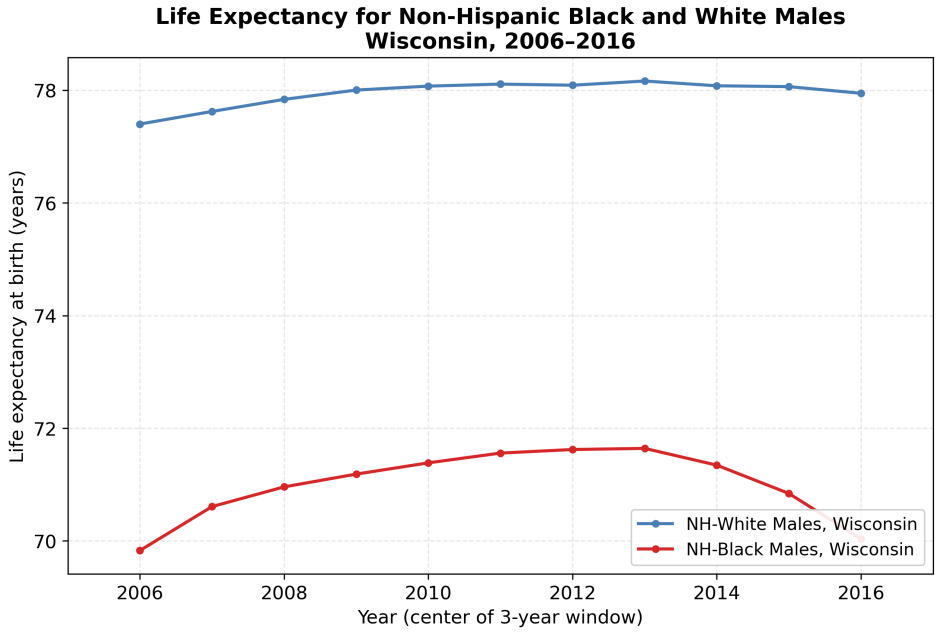

Figure B1. Life expectancy at birth for non-Hispanic Black and White males, Wisconsin statewide, 2006--2016 (3-year pooled estimates).

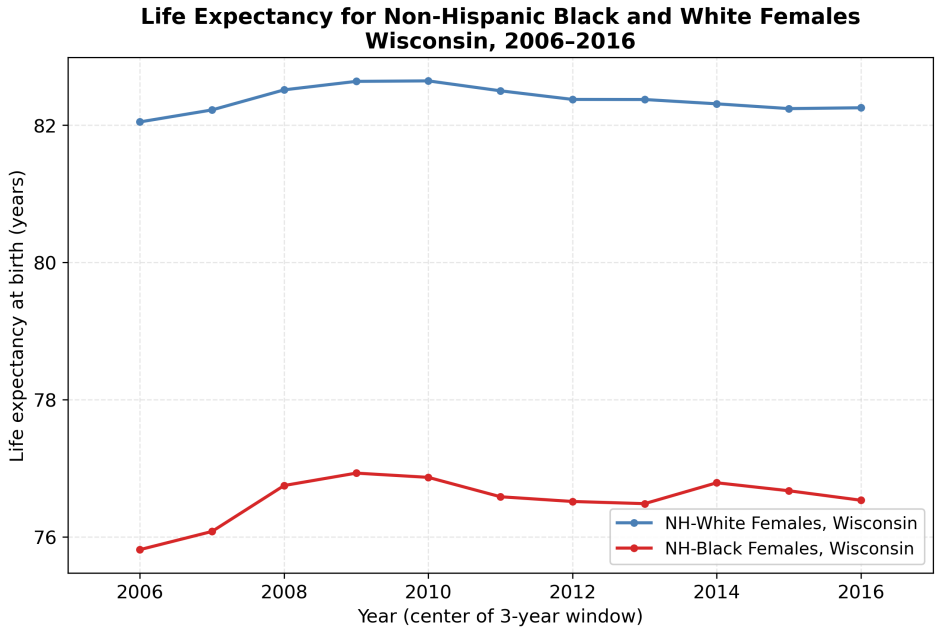

Figure B2. Life expectancy at birth for non-Hispanic Black and White females, Wisconsin statewide, 2006--2016 (3-year pooled estimates).

Appendix C: County Concentration of Black Population by State

Wisconsin's concentration of its Black population in Milwaukee County is not unusual. Many states have a single county or equivalent that holds the largest share of the state's Black population. Table C1 shows, for each state and the District of Columbia, the county (or county-equivalent) with the largest share of the state's Black population and that share. Wisconsin ranks among the states where this concentration is high: Milwaukee County holds about 67 percent of the state's Black population. Several states show even higher concentration (e.g., Nevada 94%, Hawaii 89%, Rhode Island 87%, Arizona 77%, Illinois 67%). Thus the compositional challenge for interpreting statewide disparity statistics is widespread, not unique to Wisconsin.

Table C1. County (or county-equivalent) with largest share of state's Black population and approximate share. Source: See project documentation.

State	County (largest share)	Share (%)
Alabama	Jefferson County Alabama	21.7%
Alaska	Anchorage Municipality Alask	66.1%
Arizona	Maricopa County Arizona	77.3%
Arkansas	Pulaski County Arkansas	32.6%
California	Los Angeles County Californi	35.5%
Colorado	Arapahoe County Colorado	30.7%
Connecticut	Capitol Planning Region Conn	32.1%
Delaware	New Castle County Delaware	66.2%
District of Columbia	District of Columbia	100.0%
Florida	Broward County Florida	16.4%
Georgia	Fulton County Georgia	13.7%
Hawaii	Honolulu County Hawaii	88.7%
Idaho	Ada County Idaho	49.0%
Illinois	Cook County Illinois	67.0%
Indiana	Marion County Indiana	42.8%
Iowa	Polk County Iowa	29.8%
Kansas	Sedgwick County Kansas	25.7%
Kentucky	Jefferson County Kentucky	47.6%
Louisiana	Orleans Parish Louisiana	14.8%
Maine	Cumberland County Maine	44.2%
Maryland	Prince George's County Maryl	31.8%
Massachusetts	Suffolk County Massachusetts	31.2%
Michigan	Wayne County Michigan	49.2%
Minnesota	Hennepin County Minnesota	44.2%
Mississippi	Hinds County Mississippi	15.0%
Missouri	St. Louis County Missouri	34.9%
Montana	Yellowstone County Montana	22.9%

Nebraska	Douglas County Nebraska	66.7%
Nevada	Clark County Nevada	93.7%
New Hampshire	Hillsborough County New Hamp	52.2%
New Jersey	Essex County New Jersey	26.6%
New Mexico	Bernalillo County New Mexico	43.2%
New York	Kings County New York	26.7%
North Carolina	Mecklenburg County North Car	15.9%
North Dakota	Cass County North Dakota	47.7%
Ohio	Cuyahoga County Ohio	25.2%
Oklahoma	Oklahoma County Oklahoma	41.2%
Oregon	Multnomah County Oregon	55.2%
Pennsylvania	Philadelphia County Pennsylv	45.4%
Rhode Island	Providence County Rhode Isla	87.0%
South Carolina	Richland County South Caroli	14.8%
South Dakota	Minnehaha County South Dakot	63.3%
Tennessee	Shelby County Tennessee	44.2%
Texas	Harris County Texas	25.1%
Utah	Salt Lake County Utah	57.1%
Vermont	Chittenden County Vermont	53.3%
Virginia	Fairfax County Virginia	6.9%
Washington	King County Washington	48.8%
West Virginia	Kanawha County West Virginia	19.6%
Wisconsin	Milwaukee County Wisconsin	67.0%
Wyoming	Laramie County Wyoming	48.4%

References

Arriaga, E. E. (1984). Measuring and explaining the change in life expectancies. Demography, 21(1), 83--96.

Chiang, C. L. (1968). Introduction to Stochastic Processes in Biostatistics. Wiley.

Harper, S., MacLehose, R. F., & Kaufman, J. S. (2014). Trends in the Black-White life expectancy gap among US states, 1990--2009. Health Affairs, 33(8), 1375--1382.

Kaufman, J. S., Riddell, C. A., & Harper, S. (2019). Black and White differences in life expectancy in 4 US states, 1969--2013. Public Health Reports, 134(6), 634--642.

Kochanek, K. D., Arias, E., & Anderson, R. N. (2013). How did cause of death contribute to racial differences in life expectancy in the United States in 2010? NCHS Data Brief, No. 125.

Preston, S. H., Heuveline, P., & Guillot, M. (2001). Demography: Measuring and Modeling Population Processes. Blackwell.

Riddell, C. A., Morrison, K. T., Kaufman, J. S., & Harper, S. (2018). Trends in the contribution of major causes of death to the Black-White life expectancy gap by US state. Health & Place, 52, 85--100.

Roberts, M. T., Reither, E. N., & Lim, S. (2019). Contributors to Wisconsin's persistent Black-White gap in life expectancy. BMC Public Health, 19(1), 891.

Roesch, P. T., Saiyed, N. S., Laflamme, E., De Maio, F. G., & Benjamins, M. R. (2023). Life expectancy gaps among Black and White persons and contributing causes of death in 3 large US cities, 2018--2019. JAMA Network Open, 6(3), e233146.